ONE Clinical Contracture Management Protocol

POSSIBLE F-TAG/QMS

Range of Motion F684

- Each patient must receive and the facility must provide the necessary care and services to attain the highest practicable physical, mental, and psychosocial well-being, in accordance with the comprehensive assessment and plan of care.

F688

- Based on comprehensive assessment of a patient, the facility must ensure that a patient who enters the facility without limited range of motion does not experience reduction in range of motion unless the patient's clinical condition demonstrates that a reduction in range of motion is unavoidable
- A patient with limited range of motion receives appropriate treatment and services to increase range of motion and/or to prevent further decreases in range of motion.

THERAPIES

- PT
- OT

PURPOSE OF THERAPY

- Prevent and reduce presence of contractures and improve overall quality of life and ability to engage in daily routine.

GOAL EXAMPLES

- Patient will reduce pain associated with contracture to 2/10 with use of modalities prior to manual therapy interventions.
- Pt will increase ROM at _____ (joint) by ___ degrees in order to improve wearing tolerance of prescribed splint.
- Patient will engage in hand hygiene pre and post hand splint application with Min assist to reduce risk for skin breakdown.
- Patient will tolerate _____ splint x 4 hours without s/s of skin breakdown.
- Patient will adhere to splint wearing schedule with independence utilizing visual reminders.
- Pt will be independent with self skin inspection post splint removal.
- Pt will be 100% compliant with splint application following developed wearing schedule with trained caregiver support.
- Pt will tolerate ROM and stretching prior to splint application with 100% compliance from trained caregiver support.
- Pt will increase ankle ROM to _____ in order to tolerate sit to stand with Min assist.

TOOLS, ASSESSMENTS & IMPLEMENTATION

TOOLS & ASSESSMENTS

- ROM assessments
- Evaluate bed and WC positioning that could contribute to contracture development and/or improvement.
- Seating evaluation as necessary. Include positioning program with thorough education of nursing staff
- Rate tone using Modified Ashworth Scale

IMPLEMENTATION

MODALITIES:

- Diathermy
- Ultrasound
- E-stim
- Moist Heat

MANUAL THERAPY:

- ROM
- Tissue Mobilization

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POSSIBLE F-TAG/QMS	THERAPIES	PURPOSE OF THERAPY	GOAL EXAMPLES	TOOLS, ASSESSMENTS & IMPLEMENTATION
Quality Measures - Falls with Injury - ADL Decline - Mobility Decline			- Pt will increase shoulder ROM to _____ in order to improve independence with combing hair to Mod assist. - Pt will increase elbow ROM to _____ in order to improve self feeding to Min assist. - Pt will use a modified visual pain scale on 4/5 opportunities to communicate pain levels to caregivers.	**PNF** **PAIN MANAGEMENT ASSOCIATED WITH CONTRACTURES:** - Modalities - Manual techniques - Positioning **NEURO STRETCH TECHNIQUES:** Recommended only for patients with abnormal tone or spasticity - PROM technique which should not initiate a stretch reflex. - Designed to activate the GTO at the affected joint producing an inhibitory response. - Recommended for patients with abnormal tone or spasticity. - The Neuro Stretch technique helps specify treatment approaches by assessing the tissue and neuro aspect of a contracture **SPLINTING** - Static Splinting - Dynamic Splinting